



Use a QR Code Scanner
Application To Scan the Code

CID : 2301913910
Name : MASTER.RUDRANSH SHARAN
Age / Gender : 3 Years / Male
Consulting Dr. : C9-DR PIYUSH JAIN
Reg. Location : Cloud 9, Vashi

Collected : 19-Jan-2023 / 12:02
Reported : 19-Jan-2023 / 16:42

R
E
P
O
R
T

CBC (Complete Blood Count), Blood

<u>PARAMETER</u>	<u>RESULTS</u>	<u>BIOLOGICAL REF RANGE</u>	<u>METHOD</u>
<u>RBC PARAMETERS</u>			
Haemoglobin	12.8	11.0-14.0 g/dL	Spectrophotometric
RBC	4.88	4.0-5.2 mil/cmm	Elect. Impedance
PCV	39.1	34-40 %	Measured
MCV	80	75-87 fl	Calculated
MCH	26.2	24-30 pg	Calculated
MCHC	32.7	31-37 g/dL	Calculated
RDW	13.8	11.6-14.0 %	Calculated
<u>WBC PARAMETERS</u>			
WBC Total Count	4920	5000-15000 /cmm	Elect. Impedance
<u>WBC DIFFERENTIAL AND ABSOLUTE COUNTS</u>			
Lymphocytes	37.0	44-74 %	
Absolute Lymphocytes	1820.4	6000-9000 /cmm	Calculated
Monocytes	10.0	3-7 %	
Absolute Monocytes	492.0	200-1000 /cmm	Calculated
Neutrophils	51.0	15-65 %	
Absolute Neutrophils	2509.2	1500-8000 /cmm	Calculated
Eosinophils	1.0	1-6 %	
Absolute Eosinophils	49.2	100-1000 /cmm	Calculated
Basophils	1.0	0.1-2 %	
Absolute Basophils	49.2	20-100 /cmm	Calculated
Immature Leukocytes	-		

WBC Differential Count by Absorbance & Impedance method/Microscopy.

PLATELET PARAMETERS

Platelet Count	248000	200000-450000 /cmm	Elect. Impedance
MPV	7.8	6-11 fl	Calculated
PDW	10.9	11-18 %	Calculated

RBC MORPHOLOGY



Use a QR Code Scanner
Application To Scan the Code

CID : 2301913910
Name : MASTER.RUDRANSH SHARAN
Age / Gender : 3 Years / Male
Consulting Dr. : C9-DR PIYUSH JAIN
Reg. Location : Cloud 9, Vashi

Collected : 19-Jan-2023 / 12:02
Reported : 19-Jan-2023 / 16:42

R
E
P
O
R
T

Hypochromia	-
Microcytosis	-
Macrocytosis	-
Anisocytosis	-
Poikilocytosis	-
Polychromasia	-
Target Cells	-
Basophilic Stippling	-
Normoblasts	-
Others	Normocytic, Normochromic
WBC MORPHOLOGY	-
PLATELET MORPHOLOGY	-
COMMENT	-

Kindly correlate clinically

Specimen: EDTA Whole Blood

*Sample processed at SUBURBAN DIAGNOSTICS (INDIA) PVT. LTD Cloud 9 Lab, Vashi
*** End Of Report ***



J. Thakker
Dr. JYOT THAKKER
M.D. (PATH), DPB
Pathologist & AVP(Medical Services)



Use a QR Code Scanner
Application To Scan the Code

CID : 2301913910
Name : MASTER.RUDRANSH SHARAN
Age / Gender : 3 Years / Male
Consulting Dr. : C9-DR PIYUSH JAIN
Reg. Location : Cloud 9, Vashi

Collected : 19-Jan-2023 / 12:02
Reported : 19-Jan-2023 / 16:26

R
E
P
O
R
T

MALARIAL PARASITE (MP)

<u>PARAMETER</u>	<u>RESULTS</u>	<u>BIOLOGICAL REF RANGE</u>
MALARIAL PARASITE, EDTA WB.	Negative	Negative

Intended Use: To detect the presence of Malarial Parasite on peripheral smear.

Clinical Significance:

- Examination of the thin smears allows calculation of malaria percent parasitemia, which can be used to predict prognosis and monitor response to treatment. The percentage parasitemia represents the percentage of infected red blood cells.
- WHO considers the falciparum parasitemia to be heavy and to carry a poor prognosis when more than 5 % are parasitized

Test Interpretation:

- Negative- A single negative result does not rule out Malaria. Sensitive detection of parasitemia may require examination of several specimens.
- Positive- Depending on the morphology, different malarial parasites are identified.

Reflex Tests: Rapid malaria detection test.

Limitations of the test: Low level of parasitemia may require the examination of multiple specimens for detection.

References:

- Kanai Mukherjee Vol I / CHAP:11 PG:326
- Dace and Lewis Practical haematology, 10th edition
- District laboratory practice in tropical countries Monica Cheebrough

*Sample processed at SUBURBAN DIAGNOSTICS (INDIA) PVT. LTD Panvel Lab, Panvel East
*** End Of Report ***



J. Mujawar
Dr.IMRAN MUJAWAR
M.D (Path)
Pathologist



Use a QR Code Scanner
Application To Scan the Code

CID : 2301913910
Name : MASTER.RUDRANSH SHARAN
Age / Gender : 3 Years / Male
Consulting Dr. : C9-DR PIYUSH JAIN
Reg. Location : Cloud 9, Vashi

Collected : 19-Jan-2023 / 12:02
Reported : 19-Jan-2023 / 13:27

CRP-QUANTITATIVE TEST

PARAMETER	RESULTS	BIOLOGICAL REF RANGE	METHOD
CRP-QUANTITATIVE, Serum	6.9	1-5 mg/l	Imm.Turbidimetry

Interpretation:

CRP elevations are nonspecific and may be useful for the detection of systemic inflammatory processes like;

- 1) To assess treatment of bacterial infections with antibiotics
- 2) To differentiate between active and inactive forms of disease with concurrent infection
- 3) Postoperative monitoring & to determine the presence of postoperative complications at an early stage, such as infected wounds, thrombosis, and pneumonia.

Clinical Significance:

- 1) C-reactive protein (CRP) is an acute phase reactant, a protein made by the liver and released into the blood within a few hours after tissue injury, the start of an infection, or other cause of inflammation.
- 2) The test measures the amount of CRP in the blood and can be valuable in detecting inflammation due to acute conditions or in monitoring disease activity in chronic conditions.
- 3) In normal healthy individuals CRP is a trace protein, after onset of an acute phase response the serum CRP concentration rises rapidly and extensively. Alterations are detectable within 6 to 8 hours and the peak value is reached within 24 to 48 hours.
- 4) Levels of up to thousand fold the normal value are associated with severe stimuli such as myocardial infarction, major trauma, surgery, or malignant neoplasms.
- 5) CRP has a half-life of only a few hours, making it an ideal tool for clinical monitoring. Postoperative monitoring of CRP levels of patients indicates either the normal recovery process (decreasing levels to normal) or unexpected complications (persisting high levels).
- 6) Persistence of a high serum CRP concentration is usually a grave prognostic sign which generally indicates the presence of an uncontrolled infection.
- 7) CRP determination may replace the classical determination of Erythrocytes Sedimentation Rate (ESR), due to its prompt response to changes in disease activity and its good correlation to ESR.

Reflex Tests:

- 1) Complement
- 2) Procalcitonin

Limitations of the test:

The CRP test is not diagnostic of any condition, but it can be used together with signs and symptoms and other tests to evaluate an individual for an acute or chronic inflammatory condition.

Reference:

- 1) Wallach's Interpretation of Diagnostic Tests
- 2) CRP Kit Insert

*Sample processed at SUBURBAN DIAGNOSTICS (INDIA) PVT. LTD Cloud 9 Lab, Vashi

*** End Of Report ***



J. Thakker
Dr.JYOT THAKKER
M.D. (PATH), DPB
Pathologist & AVP(Medical
Services)



Use a QR Code Scanner
Application To Scan the Code

CID : 2301913910
Name : MASTER.RUDRANSH SHARAN
Age / Gender : 3 Years / Male
Consulting Dr. : C9-DR PIYUSH JAIN
Reg. Location : Cloud 9, Vashi

Collected : 19-Jan-2023 / 12:02
Reported : 19-Jan-2023 / 17:19

DENGUE NS1 ANTIGEN

<u>PARAMETER</u>	<u>RESULTS</u>	<u>BIOLOGICAL REF RANGE</u>	<u>METHOD</u>
Dengue NS1 Antigen, Serum	Negative (0.02)	< 1.0 ≥ 1.0	Negative Positive CLIA

Kindly note change in reference range and method w.e.f. 24/08/2022

Clinical significance:

Detection of NS1 Antigen by is valuable as it detects infection prior to seroconversion. It can be detected in serum from day 1 after onset of fever and up to 9 days. This helps in early implementation of supportive therapy & monitoring, this in turn reduces risk of complications such as Dengue hemorrhagic fever or Dengue shock syndrome.

Intended use:

Qualitative or semi-quantitative detection of Dengue virus NS1 antigen in human serum or plasma by enzyme immunoassay.

Interpretation:

1. Negative: No detectable Dengue NS1 antigen. The result does not rule out Dengue infection. An additional sample should be tested with serology in 7-14 days.
2. Equivocal: Equivocal samples should be repeated. Samples that remain equivocal after repeat testing should be repeated by an alternative method or another sample should be collected.
3. Positive: Presence of detectable Dengue NS1 antigen. Dengue serology assays should be performed on follow-up samples to confirm Dengue infection.

Reflex test: Dengue RNA detection by PCR

Limitations:

1. Negative result does not rule out Dengue infection. An additional sample should be tested with serology in 7-14 days.
2. Diagnosis of recent infection can only be established on the basis of a combination of clinical and biological data. The result obtained on a single sample does not constitute a sufficient proof for diagnosis of recent infection.

Reference: Dengue NS1 kit insert

*Sample processed at SUBURBAN DIAGNOSTICS (INDIA) PVT. LTD SDRL, Vidyavihar Lab

*** End Of Report ***



Dr. LEENA SALUNKHE
M.B.B.S, DPB (PATH)
Pathologist



Use a QR Code Scanner
Application To Scan the Code

CID : 2301913910
Name : MASTER.RUDRANSH SHARAN
Age / Gender : 3 Years / Male
Consulting Dr. : C9-DR PIYUSH JAIN
Reg. Location : Cloud 9, Vashi

Collected : 19-Jan-2023 / 12:02
Reported : 19-Jan-2023 / 17:33

R
E
P
O
R
T

URINE EXAMINATION REPORT

PARAMETER	RESULTS	BIOLOGICAL REF RANGE	METHOD
PHYSICAL EXAMINATION			
Color	Yellow	Pale Yellow	-
Reaction (pH)	7.0	4.5 - 8.0	Chemical Indicator
Specific Gravity	1.020	1.001-1.030	Chemical Indicator
Transparency	Slight hazy	Clear	-
Volume (ml)	10	-	-
CHEMICAL EXAMINATION			
Proteins	Trace	Absent	pH Indicator
Glucose	Absent	Absent	GOD-POD
Ketones	Absent	Absent	Legals Test
Blood	Absent	Absent	Peroxidase
Bilirubin	Absent	Absent	Diazonium Salt
Urobilinogen	Normal	Normal	Diazonium Salt
Nitrite	Absent	Absent	Griess Test
MICROSCOPIC EXAMINATION			
Leukocytes(Pus cells)/hpf	2-3	0-5/hpf	
Red Blood Cells / hpf	Absent	0-2/hpf	
Epithelial Cells / hpf	1-2		
Casts	Absent	Absent	
Crystals	Absent	Absent	
Amorphous debris	Absent	Absent	
Bacteria / hpf	+(>20/hpf)	Less than 20/hpf	
Others	-		

Interpretation: The concentration values of Chemical analytes corresponding to the grading given in the report are as follows:

- Protein:(1+ ~25 mg/dl, 2+ ~75 mg/dl, 3+ ~ 150 mg/dl, 4+ ~ 500 mg/dl)
- Glucose:(1+ ~ 50 mg/dl, 2+ ~100 mg/dl, 3+ ~300 mg/dl,4+ ~1000 mg/dl)
- Ketone:(1+ ~5 mg/dl, 2+ ~15 mg/dl, 3+ ~ 50 mg/dl, 4+ ~ 150 mg/dl)

Reference: Pack insert

*Sample processed at SUBURBAN DIAGNOSTICS (INDIA) PVT. LTD SDRL, Vidyavihar Lab



Dr. Leena Salunkhe
Dr.LEENA SALUNKHE
M.B.B.S, DPB (PATH)
Pathologist



Use a QR Code Scanner
Application To Scan the Code

CID : 2301913910
Name : MASTER.RUDRANSH SHARAN
Age / Gender : 3 Years / Male
Consulting Dr. : C9-DR PIYUSH JAIN
Reg. Location : Cloud 9, Vashi

Collected :
Reported :

*** End Of Report ***